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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 27-year-old primigravid woman at 14 weeks' gestation comes to the emergency department because of a 24-hour history of nausea and right-sided abdominal pain. She also has had loss of appetite for the past 2 days. She has not had vomiting, and pregnancy had been uncomplicated. Her temperature is 38.2°C (100.8°F), pulse is 94/min, respirations are 20/min, and blood pressure is 120/80 mm Hg. Fetal heart tones are heard. Abdominal examination shows right lower quadrant tenderness with no rigidity or rebound. Laboratory studies show:

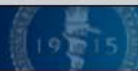
Hemoglobin	13.2 g/dL
Leukocyte count	16,500/mm ³
Segmented neutrophils	80%
Bands	10%
Lymphocytes	10%
Urine	
Specific gravity	1.030
Protein	trace
RBC	1–2/hpf
WBC	numerous
Nitrites	negative
Bacteria	none

Which of the following is the most likely diagnosis?

- ☐ A) Appendicitis
- ☐ B) Cholecystitis
- ☐ C) Chorioamnionitis
- ☐ D) Pyelonephritis
- ☐ E) Salpingitis

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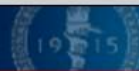
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 17-year-old girl is brought to the physician by her mother because she is concerned that her daughter has never had a menstrual period and has not had breast development. The patient is not sexually active and takes no medications. She is 124 cm (4 ft 1 in) tall and weighs 48 kg (105 lb); BMI is 31 kg/m². Her blood pressure is 115/65 mm Hg. Breast development is Tanner stage 1. The thyroid gland is not enlarged. A grade 2/6 continuous murmur is heard best over the midsternal border and back. Pelvic examination shows no abnormalities. There are no palpable masses. Measurement of which of the following serum concentrations is the most appropriate next step in diagnosis?

- ☐ A) Follicle-stimulating hormone
- ☐ B) Gonadotropin-releasing hormone
- ☐ C) Prolactin
- ☐ D) Testosterone
- ☐ E) Thyroid-stimulating hormone

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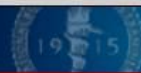
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 32-year-old woman comes to the physician because of a 6-month history of increasingly frequent pelvic cramps, pain with urination, and urinary urgency. The pain is partially relieved with urination, but she must void every 30 minutes. Menses occur at regular intervals, last 4 days, and do not aggravate her symptoms. She has no history of serious illness and takes no medications. Vital signs are within normal limits. Abdominal examination shows suprapubic tenderness. The external genitalia, vagina, and cervix appear normal. The uterus and adnexa are normal in size and nontender to palpation. The anterior vaginal wall is especially tender. Urinalysis shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Adenomyosis
- ☐ B) Endometriosis
- ☐ C) Interstitial cystitis
- ☐ D) Pelvic inflammatory disease
- ☐ E) Urinary tract infection

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Obstetrics and Gynecology Self-Assessment

A 32-year-old nulligravid woman comes to the physician because she has not had a menstrual period since she stopped taking an oral contraceptive 6 months ago. Menses had occurred at regular 28-day intervals. She also has had increased libido, increased facial acne, increased facial hair growth that requires shaving every other day, and scalp hair loss, especially on the crown. She has had an 11.3-kg (25-lb) weight gain during this period. She is sexually active with one partner, and they use condoms for contraception. She is 163 cm (5 ft 4 in) tall and weighs 86 kg (190 lb); BMI is 33 kg/m². Her vital signs are within normal limits. Physical examination shows increased development of the upper shoulder muscles. There is hair between the breasts and above the umbilicus. Pelvic examination shows the clitoris protruding completely from the clitoral hood. Ultrasonography shows a 2-cm solid mass in the right ovary. Measurement of which of the following serum hormone concentrations is most likely to be abnormal?

- ☐ A) Cortisol
- ☐ B) Dehydroepiandrosterone sulfate
- ☐ C) Prolactin
- ☐ D) Testosterone
- ☐ E) Thyroid-stimulating hormone

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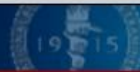
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

Four weeks after a low transverse cesarean delivery for cephalopelvic disproportion, a 27-year-old woman, gravida 1, para 1, comes to the physician because of a pulling feeling on the right side of her incision for the past 4 days; the feeling is exacerbated by movement. She was discharged on postoperative day 3. Within the past 2 weeks, she has initiated an exercise regimen to get back into shape and has resumed sexual activity. She is 168 cm (5 ft 6 in) tall and weighs 82 kg (180 lb); BMI is 29 kg/m². Her temperature is 37°C (98.6°F). The abdomen is soft and nontender without rebound in the right and left upper quadrants. There is tenderness to deep palpation just lateral to the right and left aspects of the abdominal incision; it is greater on the left than on the right. The surgical wound is clean, dry, and intact. Two shotty inguinal lymph nodes are palpated in the right inguinal region. The uterus is nontender on bimanual examination. Which of the following is the most likely explanation for this patient's discomfort?

- ☐ A) Diverticulitis
- ☐ B) Endomyometritis
- ☐ C) Gastroenteritis
- ☐ D) Incisional seroma
- ☐ E) Normal postoperative course

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National Board of Medical Examiners
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A previously healthy 42-year-old woman comes to the physician because of a 6-month history of increasingly heavy menstrual periods and a 2-month history of prolonged flow. Examination shows a slightly enlarged, irregular, smooth, freely mobile uterus. Abdominal ultrasonography shows a 4-cm leiomyoma uteri. Which of the following is the most likely type of leiomyoma?

- ☐ A) Intramural
- ☐ B) Parasitic
- ☐ C) Pedunculated
- ☐ D) Submucosal
- ☐ E) Subserosal

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Obstetrics and Gynecology Self-Assessment

A 27-year-old primigravid woman at 34 weeks' gestation comes to the physician because of a 1-day history of severe anxiety, sweating, and a rapid heartbeat. She is oriented to person and time but not to place. Her temperature is 37.8°C (100°F), pulse is 130/min, respirations are 27/min, and blood pressure is 160/100 mm Hg. The thyroid gland is nontender and diffusely enlarged on palpation. Deep tendon reflexes are 4+; there is 4+ clonus at the ankles. Laboratory studies show:

Hemoglobin	13 g/dL
Hematocrit	39%
Leukocyte count	12,000/mm ³
Platelet count	250,000/mm ³

In addition to β -adrenergic blocking agent therapy, which of the following is the most appropriate next step in management?

- ☐ A) Cephalosporin therapy
- ☐ B) Levothyroxine therapy
- ☐ C) Potassium iodide therapy
- ☐ D) Propylthiouracil therapy
- ☐ E) Irradiation of the thyroid gland

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Obstetrics and Gynecology Self-Assessment

A 32-year-old woman, gravida 5, para 4, at 18 weeks' gestation comes to the physician for a routine prenatal visit. She is Rh-negative. Her previous pregnancies required cesarean deliveries between 33 and 35 weeks' gestation for premature labor and breech presentation. Deliveries were otherwise uncomplicated. She received Rh₀(D) immune globulin during each pregnancy. She has no other history of serious illness, and her current pregnancy has been uncomplicated. Her father has hypertension, and her mother has type 2 diabetes mellitus. Her pulse is 68/min, and blood pressure is 110/60 mm Hg. Physical and pelvic examinations show no abnormalities. Ultrasonography shows an intrauterine pregnancy of a single fetus with normal anatomy in a breech presentation; the uterus is bicornuate. This patient is at increased risk for which of the following obstetric complications?

- ☐ A) Abruptio placentae
- ☐ B) Polyhydramnios
- ☐ C) Preeclampsia
- ☐ D) Preterm labor and delivery
- ☐ E) Rh isoimmunization

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A previously healthy 32-year-old primigravid woman at 10 weeks' gestation comes to the physician because of a 5-day history of nausea and vomiting and decreased appetite. She is unable to keep solids or liquids down. She has had no fever, chills, sweating, abdominal pain, or vaginal bleeding. She has no known drug allergies. Her temperature is 37°C (98.6°F), pulse is 95/min, respirations are 14/min, and blood pressure is 100/65 mm Hg. Examination shows a uterus consistent in size with a 10-week gestation. Laboratory studies show:

Hemoglobin	11.5 g/dL
Leukocyte count	8500/mm ³
Platelet count	168,000/mm ³
Serum	
Na ⁺	130 mEq/L
Cl ⁻	95 mEq/L
K ⁺	2.8 mEq/L
HCO ₃ ⁻	30 mEq/L
Urea nitrogen	10 mg/dL
Creatinine	1.1 mg/dL
Urine	
Specific gravity	1.025
Glucose	negative
Protein	negative
Ketones	large
WBC	none
RBC	none
Casts	none

Serum studies of thyroid-stimulating hormone and free thyroxine concentrations are pending. Which of the following is the most appropriate next step in management?

- ☐ A) Outpatient oral hydration and antibiotic therapy
- ☐ B) Outpatient oral hydration and antiemetic therapy
- ☐ C) Admission to the hospital for intravenous hydration only
- ☐ D) Admission to the hospital for intravenous hydration and antibiotic therapy
- ☐ E) Admission to the hospital for intravenous hydration and parenteral antiemetic therapy
- ☐ F) Admission to the hospital for oral hydration and antibiotic therapy

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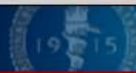
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

Eighteen hours after an uncomplicated cesarean delivery for breech presentation, a 23-year-old woman, gravida 1, para 1, has fever. Her temperature is 38°C (100.4°F), pulse is 94/min, respirations are 18/min, and blood pressure is 112/74 mm Hg. Decreased breath sounds are heard bilaterally with no crackles or rhonchi. Cardiac examination shows a regular rhythm with no murmurs, rubs, or clicks. The fundus is nontender and 2 cm below the umbilicus. The incision site is dry, intact, and mildly tender. Examination of the lower extremities shows 2+ pitting edema to the midcalves bilaterally with no cyanosis or clubbing. Foley and right intravenous antecubital catheters are in place. Which of the following is the most likely diagnosis?

- ☐ A) Atelectasis
- ☐ B) Endometritis
- ☐ C) Pyelonephritis
- ☐ D) Thrombophlebitis
- ☐ E) Wound infection

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Obstetrics and Gynecology Self-Assessment

Prior to discharge from the hospital following an uncomplicated delivery, a 30-year-old woman, gravida 1, para 1, states that she would like to resume the combination oral contraceptive she took prior to pregnancy. She plans to breast-feed for 6 months. In considering the combination of breast-feeding and this method of contraception, the physician should counsel the patient about which of the following potential problems?

- ☐ A) Decreased effectiveness in preventing pregnancy
- ☐ B) Decreased protein content in breast milk
- ☐ C) Delayed uterine involution
- ☐ D) Increased incidence of urinary tract infection
- ☐ E) Increased incidence of mastitis

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Obstetrics and Gynecology Self-Assessment

A 67-year-old woman comes to the physician because of moderate vulvar itching for 2 years. She is otherwise healthy and takes no medications. Vital signs are within normal limits. Pelvic examination shows three 1-cm areas of white epithelium over the left labium majus. There is no inguinal adenopathy or vulvovaginal discharge. Which of the following is the most appropriate next step in management?

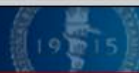
- ☐ A) MRI of the pelvis
- ☐ B) Cytologic examination of a lesion scraping
- ☐ C) Application of corticosteroid cream
- ☐ D) Application of testosterone cream
- ☐ E) Punch biopsy of the affected areas
- ☐ F) Laser ablation of the affected areas



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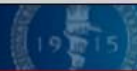
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 57-year-old woman comes to the physician because of small blood stains on her underwear during the past 6 months. Menopause occurred 5 years ago, and she has not received hormone therapy. She reports that sexual intercourse is painful. She has not had any urinary or gastrointestinal symptoms. Examination shows atrophic, friable vaginal mucosa; there is scant blood in the vaginal canal. The cervix appears normal. Bimanual examination shows a normal-sized uterus. Which of the following is the most likely cause of this patient's symptoms?

- ☐ A) Cystitis
- ☐ B) Endometrial hyperplasia
- ☐ C) Hypoestrogenic state
- ☐ D) Internal hemorrhoids
- ☐ E) Vaginal neoplasia

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Obstetrics and Gynecology Self-Assessment

A previously healthy 27-year-old woman, gravida 2, para 1, at 36 weeks' gestation comes to the physician because of a 2-hour history of intermittent vaginal bleeding. She has received no prenatal care. The fundal height is 35 cm. The fetal heart rate is 135/min. Examination of the lower genital tract and cervix shows the bleeding to be of uterine origin. Laboratory studies are within the reference range. Her blood group is O, Rh-negative, and antibody screening is negative. Fetal nonstress testing is reactive, and fetal biophysical profile score is 8/8. Which of the following is the most appropriate next step in management?

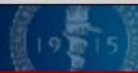
- ☐ A) Coagulation studies
- ☐ B) Measurement of fetal hemoglobin concentration
- ☐ C) Contraction stress test
- ☐ D) Administration of betamethasone
- ☐ E) Administration of Rh₀(D) immune globulin



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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A previously healthy 22-year-old woman comes to the emergency department because of increasing vaginal bleeding during the past 2 days. Her last menstrual period was 8 weeks ago. Several home pregnancy tests have yielded positive results. Her only medication is a daily prenatal vitamin. She is 165 cm (5 ft 5 in) tall and weighs 68 kg (150 lb); BMI is 25 kg/m². Her temperature is 37°C (98.6°F), pulse is 90/min, respirations are 20/min, and blood pressure is 110/60 mm Hg. Pelvic examination shows normal-appearing external genitalia and cervix; there is blood in the vaginal vault. Bimanual examination shows a soft, nontender uterus consistent in size with a 10-week gestation. Her hemoglobin concentration is 11.7 g/dL, hematocrit is 35%, and serum β -hCG concentration is 554,367 mIU/mL. Serum electrolyte concentrations are within the reference ranges. Transvaginal ultrasonography shows an enlarged uterus with no fetus; the endometrial cavity is filled with scattered hyperechoic material; both ovaries have small, 2- to 3-cm, simple cysts. Which of the following is the most appropriate next step in management?

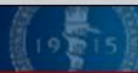
- ☐ A) Administration of methotrexate
- ☐ B) Administration of misoprostol
- ☐ C) Chorionic villus sampling
- ☐ D) Hysterectomy
- ☐ E) Suction and curettage



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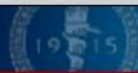
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

An 11-year-old girl is brought to the physician for a well-child examination. She has had recent mood changes, and her mother is concerned that her daughter's menstrual periods will start soon. The mother's menarche occurred at the age of 14 years. The patient is at the 75th percentile for height and 90th percentile for weight. Breast and pubic hair development is Tanner stage 3. Examination shows no abnormalities. The patient's recent pubic hair development is most predictive of which of the following?

- ☐ A) Delayed growth spurt
- ☐ B) Increased risk for breast cancer
- ☐ C) Increased risk for polycystic ovarian syndrome
- ☐ D) Menarche is imminent
- ☐ E) Precocious puberty

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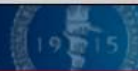
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Obstetrics and Gynecology Self-Assessment

A 27-year-old primigravid woman at 33 weeks' gestation comes to the physician for a routine prenatal visit. She has had systemic lupus erythematosus for 8 years but has been in remission for the past year. Her pregnancy has been uncomplicated except for a fundal height that began to lag 2 months ago. Ultrasonography at 20 weeks' gestation showed a fetus that had a normal anatomy and was consistent in size with gestational age. The patient's blood pressure today is 100/62 mm Hg. Pelvic examination shows no abnormalities. The fundal height is 26 cm. There is no ferning on light microscopy of vaginal secretions. Ultrasonography shows oligohydramnios and a fetus consistent in size with a 30-week gestation. Urinalysis shows no protein. Which of the following is the most likely cause of the oligohydramnios?

- ☐ A) Fetal renal agenesis
- ☐ B) Fetal urinary obstruction
- ☐ C) Intrauterine fetal infection
- ☐ D) Ruptured membranes
- ☐ E) Uteroplacental insufficiency

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Three days after a cesarean delivery at term because of failure to progress, a 27-year-old woman has a temperature of 38.8°C (101.8°F) and mild pain with urination. She has not had urinary urgency or frequency. She is bottle-feeding. Physical examination shows a clean intact incision site with no erythema. The lungs are clear to auscultation. The breasts are tense, erythematous, and tender. The uterus is firm, nontender, and consistent in size with a 20-week gestation. Laboratory studies show:

Hemoglobin	10.5 g/dL
Leukocyte count	6500/mm ³ with a normal differential
Urine	
RBC	10–15/hpf
WBC	1–2/hpf

Which of the following is the most likely diagnosis?

- ☐ A) Breast engorgement
- ☐ B) Cystitis
- ☐ C) Endometritis
- ☐ D) Mastitis
- ☐ E) Wound cellulitis



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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 20-year-old primigravid woman at 40 weeks' gestation is admitted to the hospital in labor. The cervix is 4 cm dilated; the vertex is at 0 station. Two liters of lactated Ringer solution are administered. An epidural catheter is placed, and a test dose of lidocaine and epinephrine is injected. She immediately has tinnitus and a metallic taste in her mouth. Her pulse now is 110/min, and blood pressure is 140/100 mm Hg. Which of the following is the most likely cause of these findings?

- ☐ A) Anesthetic-induced anaphylactic reaction
- ☐ B) Epidural administration of anesthetic
- ☐ C) Intravascular injection of anesthetic
- ☐ D) Mislabeling of the anesthetic agent



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A 20-year-old woman comes to the physician because of a 3-year history of mild-to-moderate hair growth over her face, breasts, and lower abdomen that has become slightly worse during the past 2 years. She has no history of serious illness and takes no medications. Menses occur at regular 28-day intervals. Examination shows excessive hair growth over the upper lip, chin, lower abdomen, and pubic area. Three days after the onset of her menstrual period, serum studies show:

Follicle-stimulating hormone	10 mIU/mL
Luteinizing hormone	12 mIU/mL
Dehydroepiandrosterone sulfate	1.5 µg/mL (N=0.5–5.4)
17α-Hydroxyprogesterone	25 ng/dL (N=20–300)
Testosterone	2.8 nmol/L (N<3.5)

Which of the following is the most likely cause of this patient's hair growth?

- ☐ A) Decreased aromatase activity
- ☐ B) Decreased progesterone concentration
- ☐ C) Increased estrogen concentration
- ☐ D) Increased 5α-reductase activity
- ☐ E) Increased testosterone concentration

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The response options for the next **2** items are the same. Select one answer for each item in the set.

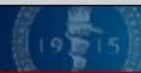
For each pregnant patient, select the most likely cause of the findings.

- ☐ A) Abruptio placentae
- ☐ B) Cervicitis
- ☐ C) Chorioamnionitis
- ☐ D) Fetal anomaly
- ☐ E) Idiopathic preterm labor
- ☐ F) Incompetent cervix
- ☐ G) Placenta previa
- ☐ H) Polyhydramnios
- ☐ I) Pyelonephritis

A 19-year-old primigravid woman at 31 weeks' gestation is admitted to the hospital because of intense uterine contractions every 1 to 2 minutes for 2 hours. The uterus is firm and tender. The fetal heart rate is 165/min. There is dark blood flowing from the vagina, and the cervix is completely effaced and 7 cm dilated; the vertex is at 0 station. Fundal height is 30 cm.

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Obstetrics and Gynecology Self-Assessment

For each pregnant patient, select the most likely cause of the findings.

- ☐ A) Abruptio placentae
- ☐ B) Cervicitis
- ☐ C) Chorioamnionitis
- ☐ D) Fetal anomaly
- ☐ E) Idiopathic preterm labor
- ☐ F) Incompetent cervix
- ☐ G) Placenta previa
- ☐ H) Polyhydramnios
- ☐ I) Pyelonephritis

A 23-year-old primigravid woman at 32 weeks' gestation is admitted to the hospital because of irregular uterine contractions for 3 hours. Her temperature is 38.2°C (100.8°F). The uterus is moderately tender to palpation. The fetal heart rate is 170/min. The cervix is 80% effaced and 2 cm dilated; the vertex is at -1 station. Fundal height is 31 cm. There is a watery vaginal discharge that tests positive to nitrazine.



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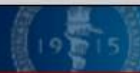
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 23-year-old woman comes for a follow-up examination 3 weeks after being diagnosed with a urinary tract infection. Treatment with trimethoprim-sulfamethoxazole has relieved her symptoms. This is her third urinary tract infection over the past year. She has no history of serious illness. She was married 3 months ago. Her temperature is 37°C (98.6°F). Examination shows no abdominal or flank tenderness. Urinalysis is within normal limits. Daily administration of which of the following is the most appropriate prophylaxis against recurrence of this condition?

- ☐ A) Dicloxacillin
- ☐ B) Penicillin
- ☐ C) Phenazopyridine
- ☐ D) Propantheline
- ☐ E) Trimethoprim-sulfamethoxazole

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A 15-year-old girl is brought to the physician because of a 1-week history of constant severe abdominal pain. During the past year, she has had 10 episodes of cramps; each episode lasts 3 to 5 days. She has no history of serious illness and takes no medications. She has never had a menstrual period. She is sexually active with one male partner, and they do not use contraception. She is at the 80th percentile for height and weight. Her temperature is 37.8°C (100°F), pulse is 120/min, respirations are 16/min, and blood pressure is 90/50 mm Hg. Breast and genital development is Tanner stage 5. Abdominal examination shows moderate tenderness. A mass is palpated in the suprapubic region at the midline. Pelvic examination shows normal-appearing external genitalia and lower vagina. The cervix cannot be visualized because of bluish bulging vaginal tissue that obscures the upper vagina. A urine pregnancy test is negative. Which of the following is the most likely diagnosis?

- ☐ A) Adenomyosis
- ☐ B) Ectopic pregnancy
- ☐ C) Hematocolpos
- ☐ D) Leiomyomata uteri
- ☐ E) Ruptured corpus luteum cyst

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A previously healthy 17-year-old girl comes to student health services because of moderately severe pelvic pain with nausea and vomiting during menses since menarche at the age of 13 years. The symptoms begin soon after the onset of menses and last for 48 hours. Ibuprofen provides moderate relief of her symptoms. She has not had fever or a change in bowel movements. Menses occur at regular 26- to 28-day intervals. She has never been sexually active. Examination, including pelvic examination, shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Adenomyosis
- ☐ B) Endometriosis
- ☐ C) Irritable bowel syndrome
- ☐ D) Pelvic inflammatory disease
- ☐ E) Primary dysmenorrhea

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A 26-year-old woman, gravida 3, para 0, aborta 3, comes to the physician because of three spontaneous first-trimester abortions during the past 4 years. She has had no previous evaluation. She has a history of recurrent urinary tract infections since childhood. An intravenous pyelography 16 years ago showed a single left kidney. Pelvic examination today shows a palpable uterus and an easily palpable left ovary. Which of the following is the most likely diagnosis?

- ☐ A) Autoimmune disorder
- ☐ B) Congenital uterine anomalies
- ☐ C) Intrauterine synechiae
- ☐ D) Müllerian agenesis
- ☐ E) Polycystic ovarian syndrome
- ☐ F) Premature ovarian failure
- ☐ G) Prolactin-secreting adenoma
- ☐ H) Short luteal phase
- ☐ I) Submucosal leiomyomata uteri
- ☐ J) Tuberculous salpingitis

27.

Exam Section

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A previously healthy 18-year-old woman comes to the physician because of a 1-day history of fever, nausea and vomiting, and diarrhea with an associated diffuse rash. She has no history of serious illness or surgical procedures. Menses occur at regular 30-day intervals. Her last menstrual period started 3 days ago. She uses tampons and sanitary pads during menstrual periods. She is sexually active with one male partner and recently started using a contraceptive patch. She appears ill. Her temperature is 39°C (102.2°F), pulse is 96/min, respirations are 22/min, and blood pressure is 90/60 mm Hg. Physical examination shows a diffuse, erythematous, maculopapular rash over the perineum and thighs. Pelvic examination shows cervical motion, uterine, and bilateral adnexal tenderness with no masses. Which of the following is the most likely causal organism?

- ☐ A) *Chlamydia trachomatis*
- ☐ B) *Haemophilus ducreyi*
- ☐ C) Herpes simplex virus
- ☐ D) *Neisseria gonorrhoeae*
- ☐ E) *Staphylococcus aureus*

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28.

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■ Mark

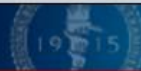
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A healthy 18-year-old nulligravid woman comes to the physician for a routine health maintenance examination. She states that she typically has pain in one of the adnexal regions that occurs during days 13 and 14 of her menstrual cycle. She describes the pain as brief and sharp. Menarche was at the age of 13 years, and menses occur at regular 28-day intervals with moderate flow for the first 2 days. She has never been sexually active. Physical and pelvic examinations show no abnormalities. Which of the following is the most appropriate next step in management?

- ☐ A) Reassurance
- ☐ B) Testing for *Neisseria gonorrhoeae* and *Chlamydia trachomatis*
- ☐ C) Pelvic ultrasonography
- ☐ D) Leuprolide therapy
- ☐ E) Laparoscopy

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A 32-year-old woman, gravida 2, para 1, at 26 weeks' gestation is brought to the emergency department 2 hours after she was awakened by the worst pain she has ever had. The pain started on the left side of her back and radiated into the left inguinal area and left labium. At the onset, she had sweating, nausea, and several episodes of vomiting. She also felt faint while attempting to urinate and could not completely empty her bladder. She only feels comfortable while she is ambulating. Her pregnancy had been uncomplicated. Her temperature is 37°C (98.6°F), pulse is 122/min, respirations are 12/min, and blood pressure is 110/65 mm Hg. Examination shows moderate tenderness in the left back and flank. There is no gross hematuria. Which of the following is the most likely diagnosis?

- ☐ A) Acute porphyria
- ☐ B) Aortic aneurysm
- ☐ C) Appendicitis
- ☐ D) Bowel perforation
- ☐ E) Diverticulitis
- ☐ F) Intussusception
- ☐ G) Pancreatitis
- ☐ H) Pyelonephritis
- ☐ I) Ruptured ectopic pregnancy
- ☐ J) Ruptured ovarian cyst
- ☐ K) Ruptured uterus
- ☐ L) Ureterolithiasis

30.

Exam Section

■ Mark

National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 27-year-old Filipino woman at 10 weeks' gestation comes for her first prenatal visit. Her pregnancy has been complicated by fatigue and nausea. She has not had fever, bleeding, or rashes. She has a 15-year history of anemia. Her hematocrit has remained 28% to 29% during the past 7 years despite iron supplementation. She has never been hospitalized. Menarche was at the age of 13 years. Menses occur at regular 28-day intervals, last 5 days, and diminish in flow after the first day. Five years ago, she visited her family in Manila and remembers becoming ill with flu-like symptoms during that visit. Examination shows a uterus consistent in size with a 10-week gestation. Which of the following is the most appropriate next step in management?

- ☐ A) Test of the stool for occult blood
- ☐ B) Hemoglobin electrophoresis
- ☐ C) Triple screening
- ☐ D) Erythropoietin therapy
- ☐ E) Interferon alfa therapy
- ☐ F) Chorionic villus sampling

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 57-year-old woman comes to the physician for a routine health maintenance examination. She has hypertension, type 2 diabetes mellitus, and generalized anxiety disorder. Her great-aunt has a history of breast cancer. The patient has been receiving hormone therapy with conjugated estrogen and medroxyprogesterone daily since menopause 5 years ago. Additional medications include hydrochlorothiazide, metformin, and various herbal medications. Her blood pressure is 136/84 mm Hg. Examination of the breasts shows no masses or nipple discharge. Which of the following historical findings is the greatest risk factor for breast cancer in this patient?

- ☐ A) Family history of breast cancer
- ☐ B) Herbal medications
- ☐ C) Hormone therapy
- ☐ D) Hypertension
- ☐ E) Type 2 diabetes mellitus

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32.

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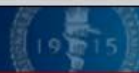
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 24-year-old primigravid woman at 30 weeks' gestation is admitted to the hospital because of bright red vaginal bleeding that she first noticed as spotting on her underwear 12 hours ago after sexual intercourse. Since then, the bleeding has increased, and she has had to wear a sanitary pad. Her pregnancy has been otherwise uncomplicated. Ultrasonography at 20 weeks' gestation showed a fundal placenta. Her pulse is 64/min, and blood pressure is 110/60 mm Hg. The uterine fundus is nontender; the fundal height is 30 cm. The fetal heart rate is 150/min with moderate variability. External fetal monitoring shows two contractions during the next hour. Which of the following is the most likely cause of this patient's bleeding?

- ☐ A) Abruptio placentae
- ☐ B) Cervical cancer
- ☐ C) Cervical trauma
- ☐ D) Placenta previa
- ☐ E) Preterm labor

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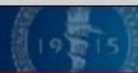
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 42-year-old woman, gravida 3, para 3, comes to the physician for a routine examination. Over the past year, menses have occurred at irregular 2- to 3-month intervals and have lasted 7 to 21 days. Her last menstrual period was 6 weeks ago. She has type 2 diabetes mellitus treated with metformin. She is 168 cm (5 ft 6 in) tall and weighs 88 kg (195 lb); BMI is 32 kg/m². Physical examination shows no other abnormalities. Pelvic examination shows an irregular enlarged uterus measuring 12 × 8 × 6 cm. An endometrial biopsy specimen shows atypical complex hyperplasia. Which of the following is the strongest predisposing factor for this patient's condition?

- ☐ A) Age
- ☐ B) Anovulation
- ☐ C) Leiomyomata uteri
- ☐ D) Metformin therapy
- ☐ E) Multiparity

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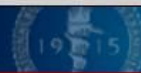
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A previously healthy 32-year-old woman comes to the physician because of a 2-month history of dull right-sided pelvic pain. The pain is most severe during menses. Her last menstrual period was 3 weeks ago. She takes no medications. Examination shows fullness in the right adnexa and tenderness to palpation. Urine β -hCG testing is negative. Pelvic ultrasonography shows a 5-cm simple cyst in the right adnexa. She says she is anxious about paying for treatment because her medical insurance expires in 2 weeks. Which of the following is the most appropriate next step in management?

- ☐ A) Reassurance and schedule a follow-up visit in 1 year
- ☐ B) Oral contraceptive therapy and a second pelvic examination in 6 weeks
- ☐ C) Ultrasonography-guided drainage of the cyst
- ☐ D) Laparoscopy and oophorectomy
- ☐ E) Laparotomy and oophorectomy
- ☐ F) Total abdominal hysterectomy and bilateral salpingo-oophorectomy

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 47-year-old woman comes to the physician 2 weeks after she found a lump in her left breast during self-examination. She started estrogen replacement therapy 3 months ago and has had breast engorgement since that time. Examination of the left breast shows a 2-cm, tense, mobile, cyst-like structure at the 11-o'clock position. There is no breast discharge or palpable axillary nodes. The right breast is normal. Mammography 3 months ago showed normal findings. Which of the following is the most appropriate next step in management?

- ☐ A) Repeat mammography
- ☐ B) Decrease the dose of estrogen
- ☐ C) Discontinue estrogen replacement therapy
- ☐ D) Fine-needle aspiration biopsy of the cyst
- ☐ E) Excisional biopsy



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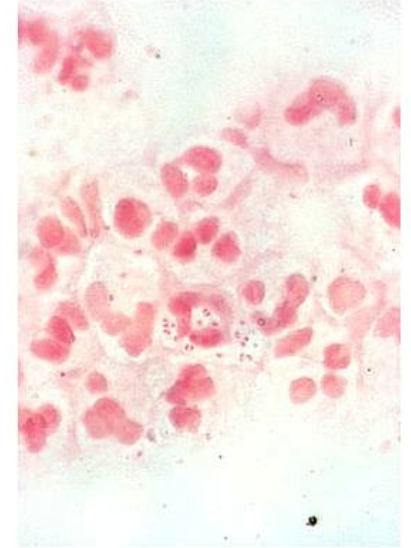
36.

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 21-year-old woman has had vaginal discharge and discomfort for 2 days. She has been sexually active with a new partner for 2 weeks; he is asymptomatic. Pelvic examination shows a purulent cervical discharge and cervical motion tenderness. A Gram stain of the discharge is shown. Which of the following is the most likely causal organism?

- ☐ A) *Actinomyces israelii*
- ☐ B) *Chlamydia trachomatis*
- ☐ C) Herpes simplex virus 2
- ☐ D) Human papillomavirus
- ☐ E) *Neisseria gonorrhoeae*



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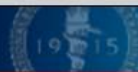
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 25-year-old woman who is HIV positive comes to the physician because of a thin, clear vaginal discharge and increased urinary frequency over the past 2 weeks. Her last menstrual period was 6 weeks ago. Menses had occurred at regular 28-day intervals. Her medications include antiretroviral agents, but she has not been compliant with the regimen. She uses condoms occasionally for contraception. Examination shows a friable cervix. The uterus is slightly enlarged, and the adnexa are normal bilaterally. Which of the following is the most likely diagnosis?

- ☐ A) Cervical cancer
- ☐ B) Cervical dysplasia
- ☐ C) Coagulopathy
- ☐ D) Endometrial hyperplasia
- ☐ E) Hypothalamic amenorrhea
- ☐ F) Luteoma
- ☐ G) Polycystic ovarian syndrome
- ☐ H) Pregnancy
- ☐ I) Urinary tract infection

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A 22-year-old woman comes to the physician because of a 3-day history of pain with urination, intense vaginal itching, and a watery discharge. She has no history of serious illness and takes no medications. She has smoked one pack of cigarettes daily for 5 years. She is sexually active and does not use contraception. Her temperature is 37°C (98.6°F). Abdominal examination shows no abnormalities. Genitourinary examination shows erythema of the vulva and vagina; there is a yellow-gray frothy discharge with a mildly fishy odor. The cervix appears normal. The pH of the vaginal discharge is 5. A wet mount preparation of the discharge is most likely to show which of the following?

- ☐ A) Flagellated protozoa
- ☐ B) Intracellular diplococci
- ☐ C) Multinucleated giant cells
- ☐ D) Pseudohyphae
- ☐ E) Spirochetes

39.

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 23-year-old primigravid woman at 30 weeks' gestation is brought to the emergency department because of headache, blurred vision, and constant right upper quadrant abdominal pain for 12 hours. Her pulse is 92/min, respirations are 14/min, and blood pressure is 138/95 mm Hg. Examination shows moderate edema of the face and fingers. Deep tendon reflexes are 3+. Laboratory studies show:

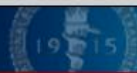
Platelet count	40,000/mm ³
Serum	
AST	1200 U/L
ALT	365 U/L
Lactate dehydrogenase	1954 U/L

Which of the following is the most likely diagnosis?

- ☐ A) Abruptio placentae
- ☐ B) Hepatitis
- ☐ C) Idiopathic cholestasis of pregnancy
- ☐ D) Immune thrombocytopenic purpura
- ☐ E) Severe preeclampsia

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 32-year-old woman, gravida 3, para 2, at 20 weeks' gestation comes to the physician for a routine prenatal visit. Her pregnancy has been uncomplicated. Examination shows a uterus consistent in size with a 20-week gestation. Ultrasonography shows skeletal dysplasia of the fetus consistent with heterozygous achondroplasia. The patient should be counseled that the inheritance pattern in this disorder is which of the following?

- ☐ A) Autosomal dominant
- ☐ B) Autosomal recessive
- ☐ C) Mitochondrial
- ☐ D) Multifactorial
- ☐ E) X-linked dominant
- ☐ F) X-linked recessive

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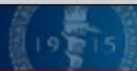
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 22-year-old woman comes to the physician because of a second episode of painful vesicular genital lesions. She says that her new sexual partner has similar lesions on his penis. If left untreated, which of the following is the most likely clinical course?

- ☐ A) Development of a severe systemic illness over the next 3 days
- ☐ B) Development of an exfoliative dermatitis within 1 week
- ☐ C) Increasing symptoms for 3 weeks, then a gradual decrease
- ☐ D) Persistence of the lesions for 3 months or more
- ☐ E) Spontaneous disappearance of the lesions within 1 week

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42.

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 32-year-old woman, gravida 5, para 4, at 21 weeks' gestation comes to the emergency department because of bright red vaginal bleeding for 4 hours. She has received no prenatal care. Vital signs are within normal limits. The fetal heart rate is 140/min. External fetal monitoring shows no contractions. Speculum examination shows bright red blood in the posterior fornix of the vagina. There is no bleeding from the cervix. Visual examination of the vagina and cervix shows no other abnormalities. Which of the following is the most appropriate next step in management?

- ☐ A) Vaginal swab for fetal fibronectin
- ☐ B) Cervical cultures
- ☐ C) Fetal ultrasonography
- ☐ D) Digital examination
- ☐ E) Cesarean delivery

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43.

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A healthy 22-year-old primigravid woman at 20 weeks' gestation comes to the physician for a routine prenatal visit. Her pregnancy has been uncomplicated. She has declined aneuploidy screening, and results of all other laboratory studies have been within the reference range. Examination shows a uterus consistent in size with a 20-week gestation. Fetal ultrasonography shows normal development of cranial structures, spine, heart, kidneys, bladder, and extremities; the abdominal organs are seen outside of the abdominal cavity without a covering membrane. The umbilical cord has three vessels, but the insertion is clearly visualized medial to the abdominal wall defect. Which of the following is the most likely diagnosis?

- ☐ A) Diaphragmatic hernia
- ☐ B) Duodenal atresia
- ☐ C) Gastroschisis
- ☐ D) Omphalocele
- ☐ E) Umbilical cord hernia



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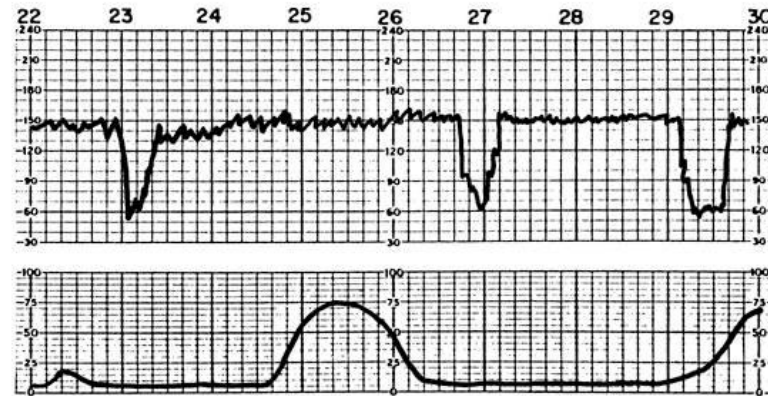
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A 42-year-old woman, gravida 8, para 7, at 42 weeks' gestation is admitted in labor. Contractions occur every 3 to 4 minutes, last 50 to 60 seconds, and have been uncomfortable for the past hour. She has the urge to push. Examination shows that the uterus extends 36 cm above the pubic symphysis. The estimated fetal weight is 3500 g (7 lb 11 oz). Fetal heart tones are 144/min. Vaginal examination shows the pooling of copious fluid. The cervix is completely dilated and effaced; the vertex is at +2 station in a left occiput anterior presentation. A fetal heart tracing is shown. Which of the following is the most likely explanation for these findings?

- ☐ A) Chorioamnionitis
- ☐ B) Fetal distress
- ☐ C) Fetal malpresentation
- ☐ D) Second stage of labor
- ☐ E) Umbilical cord compression

45.

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■ Mark

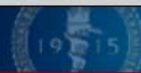
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

An 18-year-old woman comes to the physician because she has not had a menstrual period for the past year. She did not have withdrawal bleeding after a 7-day course of medroxyprogesterone. She is not sexually active and has no history of serious illness. She is 157 cm (5 ft 2 in) tall and weighs 50 kg (110 lb); BMI is 20 kg/m². Breast development is Tanner stage 2, and pubic hair development is Tanner stage 5. Pelvic examination shows a normal vagina and cervix and prepubertal-sized uterus. Serum studies show a follicle-stimulating hormone concentration of 2 mIU/mL, prolactin concentration of 15 ng/mL, and thyroid-stimulating hormone concentration of 2 µU/mL. An MRI of the brain shows no abnormalities. If left untreated over the next 10 years, this patient is at greatest risk for which of the following?

- ☐ A) Coronary artery disease
- ☐ B) Endometrial cancer
- ☐ C) Hirsutism
- ☐ D) Obesity
- ☐ E) Osteoporosis

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A 67-year-old woman comes to the physician because of a 9-month history of persistent vulvar itching despite treatment with over-the-counter zinc oxide, vitamin E, hydrocortisone cream, and miconazole. She has type 2 diabetes mellitus and hypercholesterolemia. She is 155 cm (5 ft 1 in) tall and weighs 127 kg (280 lb); BMI is 53 kg/m². Examination shows an erythematous swollen vulva, papules and pustules on the medial aspect of the thighs, and several excoriations. A KOH preparation of a scraping of a pustule shows pseudohyphae and budding yeast. Which of the following is the most likely underlying cause of this patient's failure to respond to treatment?

- ☐ A) Contact dermatitis
- ☐ B) Hypercholesterolemia
- ☐ C) Self-induced excoriations
- ☐ D) Type 2 diabetes mellitus
- ☐ E) Vulvar malignancy

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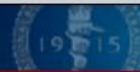
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 32-year-old woman, gravida 3, para 2, with type 2 diabetes mellitus is admitted to the hospital in labor at 38 weeks' gestation. Her first two children were born after normal spontaneous vaginal deliveries. On admission, the cervix is 2 cm dilated, the membranes are intact, and the fundal height is 42 cm. Four hours after admission, the cervix is completely dilated, and the vertex is occiput anterior and at -1 station. Over the next hour, contractions occur every 2 minutes, last 60 seconds, and are 60 mm Hg in intensity, but the cervix and station remain unchanged. Fetal heart tones are normal and reactive. One hour later, her condition remains unchanged. Which of the following is the most likely cause?

- ☐ A) Cephalopelvic disproportion
- ☐ B) Contraction of the outlet
- ☐ C) Fetal malposition
- ☐ D) Hypotonic uterine activity
- ☐ E) Shoulder dystocia

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National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

An 87-year-old woman comes to the physician because of urinary incontinence over the past 6 years. She now avoids leaving her house for fear of having loss of urine in public. The incontinence occurs with sneezing, coughing, or exertion. She underwent a hysterectomy 30 years ago. She is 160 cm (5 ft 3 in) tall and weighs 80 kg (176 lb); BMI is 31 kg/m². Examination shows an abdominal surgical scar. No abnormalities are noted. Which of the following is the most likely cause of these symptoms?

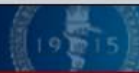
- ☐ A) Decreased external urethral sphincter tone
- ☐ B) Detrusor atony
- ☐ C) Detrusor instability
- ☐ D) Impaired urinary concentrating ability
- ☐ E) Parasympathetic denervation of the bladder
- ☐ F) Sympathetic denervation of the bladder neck



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A 32-year-old woman, gravida 1, para 1, comes to the physician because of fever and right breast tenderness for 1 day. She is breast-feeding her 14-day-old newborn. She has type 1 diabetes mellitus well controlled with insulin. Her temperature is 39.3°C (102.7°F), and pulse is 122/min. Examination shows engorgement of the breasts bilaterally. There is an erythematous, nonfluctuant, tender area in the upper outer quadrant of the right breast. Abdominal examination shows no distention or tenderness. Which of the following is the most likely diagnosis?

- ☐ A) Breast abscess
- ☐ B) Breast carcinoma
- ☐ C) Fat necrosis
- ☐ D) Fibroadenoma
- ☐ E) Lipoma
- ☐ F) Mastitis

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■ Mark

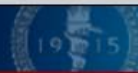
National Board of Medical Examiners
Obstetrics and Gynecology Self-Assessment

A 27-year-old woman, gravida 2, para 1, comes to the physician 3 days after an episode of bright red vaginal bleeding with no uterine contractions or cramping. She has had increasing breast size, morning sickness, and extreme fatigue. Her last menstrual period was 8 weeks ago. Examination shows a uterus consistent in size with a 6-week gestation. Vaginal ultrasonography shows normal fetal heart activity. Which of the following is the most likely diagnosis?

- ☐ A) Abruptio placentae
- ☐ B) Cervical polyp
- ☐ C) Cervicitis
- ☐ D) Hydatidiform mole
- ☐ E) Missed abortion
- ☐ F) Placenta previa
- ☐ G) Subchorionic hematoma
- ☐ H) Vaginal laceration
- ☐ I) Vasa previa
- ☐ J) Normal menses
- ☐ K) Normal pregnancy

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